

THE SOMATIC RESET SERIES

Trauma Release for Beginners

*Simple Somatic Practices You Can Do at
Home*

Disclaimer

This book is intended for educational and informational purposes only. It is not a substitute for professional medical, psychological, or psychiatric advice, diagnosis, or treatment. Always consult a qualified healthcare provider before making changes to your health routine, especially if you have a pre-existing condition.

The practices described here are general wellness approaches. Results will vary between individuals. The author and publisher accept no liability for outcomes arising from use of this material.

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What Trauma Actually Is

Trauma is not what happened to you. It's what happened inside you as a result of what happened. This distinction, from Gabor Maté, is one of the most useful reframes in modern trauma understanding — because it shifts the focus from the event to the response, and the response is where healing lives.

Big T and Little t Trauma

When most people hear the word trauma, they think of specific catastrophic events: combat, assault, serious accidents, natural disasters. This is what clinicians call big-T trauma. But the concept of small-t trauma — the accumulation of smaller, less dramatic experiences that nonetheless overwhelm the nervous system's capacity to process and integrate — has become increasingly central to understanding why so many people carry the physiological signatures of trauma without a single defining traumatic event.

Small-t trauma includes: chronic emotional neglect, persistent humiliation or criticism in childhood, unstable or unpredictable caregiving environments, experiences of powerlessness or helplessness that didn't involve

physical danger, medical procedures undergone without adequate emotional support, and many others. These experiences, particularly when they occur early and repeatedly, shape the nervous system's threat sensitivity in lasting ways.

Peter Levine, whose work on somatic experiencing is foundational to this book, describes trauma as the result of an incomplete defensive response — a threat response that was mobilised but couldn't complete its intended action. The energy remains in the body as frozen activation.

Trauma in the Body

Bessel van der Kolk's research, summarised in *The Body Keeps the Score*, has documented how trauma is stored in the body's systems — in altered stress hormone patterns, in the body map in the brain, in immune function, in posture, in the way breath moves. Brain imaging of people with PTSD shows not just changes in the emotional brain but changes in the body-sensing areas, the areas that construct our sense of physical self.

This is why talking about trauma has limits. Talking engages the verbal, cognitive parts of the brain. Trauma is stored in systems that don't have verbal access. Somatic approaches — working directly with the body, breath, movement, sensation — access these systems in

ways that conversation can't.

"Trauma is not a life sentence. It is a pattern of activation stored in the body, and the body can learn new patterns."

Safety Before Processing

This is the most important chapter in this book. Please read it before attempting any of the practices.

Why Safety Comes First

Trauma processing that happens without adequate safety does not heal trauma — it retraumatizes. The nervous system cannot process what it cannot tolerate experiencing. Pushing into trauma material before sufficient capacity has been built produces overwhelm, flooding, and often a temporary worsening of symptoms.

The practices in this book are designed for self-directed beginners. They are titrated and gentle. They are not a substitute for working with a trained trauma therapist for complex or severe presentations. If you have a history of significant trauma — particularly developmental trauma, severe PTSD, or a history of dissociation — please work with a qualified professional rather than, or in addition to, using this book.

Building a Safety Foundation

Before engaging with any trauma-focused practice, spend at least one to two weeks establishing a solid grounding and regulation foundation using the practices from earlier books in this series: consistent breathwork, grounding exercises, and at least basic sleep hygiene. These build the nervous system capacity that trauma processing requires.

Also establish a reliable way to stop: if at any point during a practice you feel overwhelmed, flooded, dissociated, or unable to function, stop the practice immediately and ground: feet on floor, eyes open, slow breath, orienting to the room by naming five visible objects. This is your emergency exit and it's important to know it clearly before you begin.

*"Go only as fast as the slowest part of you can
go."*

The Core Trauma Release Practices

These practices are drawn from Somatic Experiencing, TRE, and related body-based approaches. They are adapted for safe self-directed use.

Tracking Sensation

The most fundamental somatic skill in trauma work is tracking sensation — noticing physical sensations in the body without immediately moving to interpret or change them. This is harder than it sounds. The tendency is to move quickly from sensation to meaning: 'my chest is tight' 'something is wrong' 'I need to fix this'. Tracking interrupts this sequence at the first step.

Practice: sit quietly, close your eyes, and simply notice physical sensations in the body for five minutes. Where is there warmth? Tightness? Tingling? Heaviness? Openness? Don't analyze. Don't follow the story. Just track, the way you'd track weather through a window — observing without getting wet.

Pendulation Practice

As described in the grief chapter, pendulation involves moving attention between a site of discomfort and a resource. In trauma work, this is the core processing tool. A resource can be a physical sensation of comfort, a memory of safety, a supportive person's face, a place in the body that feels neutral or pleasant.

Practice: identify a mild, manageable sensation associated with a stressful memory (not the worst thing — start with something small). Notice it in the body: where is it? What does it feel like? Now move attention completely to the resource. Stay there until you feel some settling. Then, briefly, to the stressor sensation. Then back to resource. Repeat. Each cycle processes a small amount of the activation without overwhelming the system.

Completing the Defensive Response

Levine's approach to trauma processing often involves physically completing the defensive movement that was interrupted during the traumatic event. This is not about re-enacting the trauma — it's about allowing the body's impulse toward protective action to complete.

A simplified version: sit or lie comfortably. Bring gentle attention to a stressful memory. Notice if there's an impulse — however small — toward any particular movement. Perhaps a hand wants to push away. An arm

wants to extend. The spine wants to curl. Allow the movement to happen in slow motion, extremely slowly, with full sensory attention. Let it complete. Then pause and notice what has changed in the body.

"The body doesn't forget. But it can finish what it started."

Building a Sustainable Practice

Trauma work done well is slow. The pace that feels frustratingly slow is usually exactly right.

How Often and How Long

For self-directed trauma-oriented somatic practice, less is often more. Daily short sessions (ten to fifteen minutes) are more appropriate than long occasional sessions. The nervous system needs time to integrate between sessions.

Signs that you're moving at the right pace: you feel slightly unsettled after sessions but return to baseline within a few hours. Signs that you're moving too fast: you feel significantly worse for days after a session, or you're experiencing intrusive thoughts, sleep disruption, or dissociation. If the latter, slow down dramatically and prioritise grounding practices.

When Professional Support Is Essential

Self-directed somatic practice is appropriate for mild to moderate presentations and as a supplement to professional work. It is not appropriate as the primary

intervention for PTSD, complex developmental trauma, trauma involving violence or sexual assault, or any presentation where symptoms are severe and significantly impacting function.

A trained somatic trauma therapist offers something no book can: real-time attunement to your nervous system state, the ability to adjust the pace and direction as you go, and the co-regulatory safety of a skilled therapeutic relationship. If you have significant trauma, please find a Somatic Experiencing practitioner, an EMDR therapist, or a trauma-informed therapist. The Somatic Experiencing International website maintains a practitioner directory.

*"Healing trauma is not a heroic solo journey.
It's a relational process that often goes better
in good company."*
